

Policy on Nursing Care

«Hospital Name»

Document control

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Prepared by	Nursing Superintendent Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» ensures that nursing care is provided to patients in consonance with clinical protocols. Nursing care follows written guidance, is assigned based on clinical need and competence, is documented in an individualised nursing care plan, and is supported by appropriate equipment. Nurses are empowered to make patient care decisions within their defined scope of practice.

1. Purpose

This policy establishes requirements for providing nursing care through written guidance and clinical practice guidelines, assigning patient care based on clinical requirements and nursing competence, implementing acuity-based staffing, maintaining individualised nursing care plans, providing appropriate nursing equipment, and defining the scope of nursing practice decisions.

2. Scope

This policy applies to all nursing staff providing patient care across all wards, departments and care settings of «Hospital Name».

3. Policy standards

3.1 Written guidance and clinical practice guidelines

Nursing care is provided in accordance with written guidance. The written guidance could be in the form of a nursing manual or standard operating procedures incorporating various basic nursing practices and procedures.

Care of patients in specific clinical situations shall be guided by nursing clinical practice guidelines based on best clinical practices.

Nursing clinical care guidelines and pathways shall be reviewed annually at the minimum, and revised as appropriate.

Examples of nursing clinical practice guidelines include prevention of fall, prevention of development of pressure ulcers in an in-patient, and deep venous thrombosis risk assessment and prevention. These examples are illustrative; the organisation selects guidelines appropriate to its clinical scope.

3.2 Assignment of patient care

Assignment of patient care shall be based on the patient's clinical requirements and the competence of the nursing staff, and shall align with the guidelines laid down by regulatory and professional bodies.

3.3 Acuity-based staffing

The organisation implements acuity-based staffing — matching both the number and competence of nursing personnel to patient acuity — to improve patient outcomes.

Patient outcomes linked to acuity-based staffing may include incidence of pressure sores, falls, medication administration errors, and ventilator-associated pneumonia. The organisation selects relevant outcome indicators for its own scope of services.

3.4 Nursing care plan

Care shall be provided as per the nursing care plan, which shall be individualised as per the clinical needs of each patient. Where a patient care plan has been developed, the nursing care plan shall be aligned with it. Uniformity and continuity of care shall be practised.

Components of the nursing care plan include: Assessment; Plan of care; Implementation of care; Evaluation; and Modification of plan of care as may be required.

Documentation includes all nursing-related care and not just monitoring of vitals and documentation of medication administration. Nursing progress shall be documented in a timely manner for each patient.

3.5 Nursing equipment

There shall be an adequate number of basic nursing equipment and gadgets necessary for functioning in each designated area. Examples include nebuliser machines, glucometers, sphygmomanometers, thermometers and weighing scales — the organisation determines the appropriate equipment for its scope and patient population.

The equipment shall be appropriate for the area. For example, BP cuffs in a paediatric area shall be of appropriate size for the patient population served.

3.6 Scope of nursing practice decisions

The organisation shall define the patient care decisions that come under the scope of nursing practice.

Nurses shall be aware of their defined scope and shall be able to make appropriate nursing-related decisions in a timely manner.

4. Non-negotiable rules

- Do not provide nursing care for specific clinical situations without nursing clinical practice guidelines based on best clinical practices.
- Do not let nursing clinical care guidelines and pathways go more than one year without review.
- Do not assign patient care without considering the patient's clinical requirements and the competence of the nursing staff.
- Do not omit the nursing care plan for any patient where one is required; the plan shall be individualised as per the patient's clinical needs.
- Do not omit any of the five nursing care plan components: Assessment; Plan of care; Implementation of care; Evaluation; and Modification of plan of care as may be required.
- Do not leave a ward or department without an adequate number of nursing equipment appropriate for the area and patient population.
- Do not leave the scope of nursing practice decisions undefined or leave nurses unaware of their defined scope.

5. What we do

5.1 Written guidance and clinical practice guidelines

- Provide written guidance for nursing care — this could be a nursing manual or SOPs covering basic nursing practices and procedures.
- Guide care of patients in specific clinical situations through nursing clinical practice guidelines based on best clinical practices.
- Review nursing clinical care guidelines and pathways annually at the minimum and revise as appropriate.

- Examples of applicable guidelines include fall prevention, pressure ulcer prevention and DVT risk assessment — the organisation selects relevant guidelines for its scope.

5.2 Assignment of patient care

- Assign patient care based on the patient's clinical requirements and the competence of the nursing staff, aligned with regulatory and professional body guidelines.

5.3 Acuity-based staffing

- Implement acuity-based staffing — match the number and competence of nursing personnel to patient acuity — to improve patient outcomes.
- Monitor patient outcomes linked to acuity-based staffing and use results for improvement.

5.4 Nursing care plan

- Provide nursing care as per an individualised nursing care plan based on each patient's clinical needs; where a patient care plan exists, align the nursing care plan with it.
- Practise uniformity and continuity of care.
- Ensure every nursing care plan covers all five components: Assessment; Plan of care; Implementation of care; Evaluation; and Modification of plan of care as may be required.
- Document all nursing-related care — not just monitoring of vitals and medication administration — in a timely manner for each patient.

5.5 Nursing equipment

- Maintain an adequate number of basic nursing equipment appropriate for each ward and department and the patient population it serves.
- Ensure equipment is appropriate for the area — e.g., appropriate-size BP cuffs in paediatric areas.

5.6 Scope of nursing practice decisions

- Define the patient care decisions that come under the scope of nursing practice and ensure all nurses are aware of their defined scope.
- Ensure nurses are able to make appropriate nursing-related decisions in a timely manner.

6. Governance and responsibility

Role	Responsibility
Nursing Superintendent	Owns day-to-day implementation of nursing care processes; ensures nursing clinical practice guidelines are reviewed annually at the minimum; oversees individualised nursing care plan documentation and compliance.
Medical Superintendent	Accountable that this policy is followed and

	appropriately resourced.
Quality Coordinator	Supports audit of nursing care documentation, CPG review records and nursing equipment adequacy.

7. Quality monitoring

Monitoring area	What is monitored
Written guidance and CPG review	Nursing written guidance in place; nursing CPGs/pathways reviewed annually at the minimum and revised as appropriate
Nursing care plans	Individualised nursing care plans present and documented, covering all five components: assessment, plan of care, implementation of care, evaluation, and modification of plan of care as may be required
Nursing progress documentation	Nursing progress documented in a timely manner for each patient, covering all nursing-related care and not only vitals and medication administration
Care assignment	Patient care assignment based on clinical requirements and nursing staff competence, aligned with regulatory and professional body guidelines
Nursing equipment	Adequate and area-appropriate nursing equipment present in all wards and departments
Scope of practice	Defined scope of nursing practice decisions; nurses aware and making timely decisions

8. Training and staff acknowledgement

Nursing staff shall have access to ongoing training on nursing care processes, clinical practice guidelines and care plan documentation.

I have read the Policy on Nursing Care of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to all nursing staff, the Nursing Superintendent, ward in-charges and the Quality Coordinator.

10. Abbreviations

Abbreviation	Meaning
CPG	Clinical Practice Guideline
DVT	Deep Venous Thrombosis
SOP	Standard Operating Procedure
VAP	Ventilator-Associated Pneumonia

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.6.a	Commitment*	Sections 3.1 and 5.1 address nursing care in accordance with written guidance, nursing CPGs based on best clinical practices for specific clinical situations, and annual review of nursing CPGs/pathways at the minimum.
COP.6.b	Commitment	Sections 3.2 and 5.2 address assignment of patient care based on clinical requirements and nursing staff competence, aligned with regulatory and professional body guidelines.
COP.6.c	Achievement	Sections 3.3 and 5.3 address implementation of acuity-based staffing — matching numbers and competence to patient acuity — to improve patient outcomes.
COP.6.d	Commitment*	Sections 3.4 and 5.4 address the individualised nursing care plan aligned with the overall patient care plan where one exists; uniformity and continuity of care; all five nursing care

		plan components; and timely documentation of all nursing-related care.
COP.6.e	Commitment	Sections 3.5 and 5.5 address adequate nursing equipment in adequate numbers, appropriate for each area and patient population.
COP.6.f	Commitment	Sections 3.6 and 5.6 address the defined scope of nursing practice decisions and nurses' ability to make timely nursing decisions within that scope.

12. Required Records/Evidence Checklist

Written guidance and CPG review

- Nursing written guidance (nursing manual, SOPs, or clinical practice guidelines) covering basic nursing practices and procedures.
- Nursing clinical practice guidelines for specific clinical situations, based on best clinical practices.
- Evidence of annual review and revision of nursing CPGs and pathways at the minimum.

Nursing care plan and documentation

- Individualised nursing care plans covering all five components: assessment, plan of care, implementation of care, evaluation, and modification of plan of care as may be required.
- Evidence that the nursing care plan is aligned with the overall patient care plan where one exists.
- Nursing progress records documenting all nursing-related care in a timely manner, not limited to vitals and medication administration.

Care assignment and staffing

- Evidence that patient care assignment is based on clinical requirements and nursing staff competence, aligned with regulatory and professional body guidelines.
- Acuity-based staffing arrangements — numbers and competence matched to patient acuity — and patient outcome monitoring.

Nursing equipment

- Adequate number of basic nursing equipment present in each ward and department, appropriate for the area and patient population served.

Scope of nursing practice

- Written definition of patient care decisions within the scope of nursing practice.
- Evidence that nurses are aware of their defined scope and make timely nursing decisions.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.6.

Disclaimer

This policy reorganises the supplied COP.6 objective elements and Guidebook interpretation into plain language. Mandatory requirements and their stated modal strength have been retained. Illustrative examples, optional approaches and references have not been converted into mandatory requirements.